



Clinical Nutrition Pilot

Interim analysis for clinical review

8

patients enrolled

27

weekly reviews recorded

40

median days under care

Generated 05 Sep 2026

Single-arm pilot, no control group. Read with the limitations on the final page.



How it works

Every patient runs the same loop, and the loop repeats weekly for as long as they are on the programme.



Personalised to the patient, not to a product

Targets are set per kilogram against the correct weight basis, then split between what the patient eats and what the supplement must supply. Four formulations — standard, diabetic, anti-inflammatory and hydrolysed — are selected by the patient's own labs and symptoms.

The prescription changes as the patient does

A week of measurements produces a new set of targets and a new batch. The patient is not held on the formula they were given at enrolment — which is why the same patient can appear at different calorie targets across the dataset.

This is the workflow the pilot data below came out of. Every figure in this pack is a by-product of care being delivered, not a separate research exercise.

Safety and governance

What stops the system from producing a confident-looking number that is wrong.

136 guideline-sourced constants

Every threshold the engine uses is a named, editable constant with its source recorded — ESPEN Oncology 2021 for calorie and protein targets, KDIGO 2012 for the renal protein cap, Devine for ideal body weight, BAPEN for MUST. Changing clinical practice means editing a value, not rewriting the engine.

19 automated safety checks

Renal impairment caps protein regardless of every other rule. Neutropenia switches the diet. Low platelets restrict omega-3. Hyperkalaemia, hyponatraemia, hyperglycaemia and liver derangement each carry their own protocol. The checks run on every prescription, not on request.

A doctor signs every week

A weekly prescription cannot be approved until weight, ECOG, oral intake, supplement compliance, albumin, glucose, creatinine and urea are all present. An incomplete week is still visible — it simply cannot be signed, so a thin work-up never reaches manufacturing looking complete.

Everything is traceable

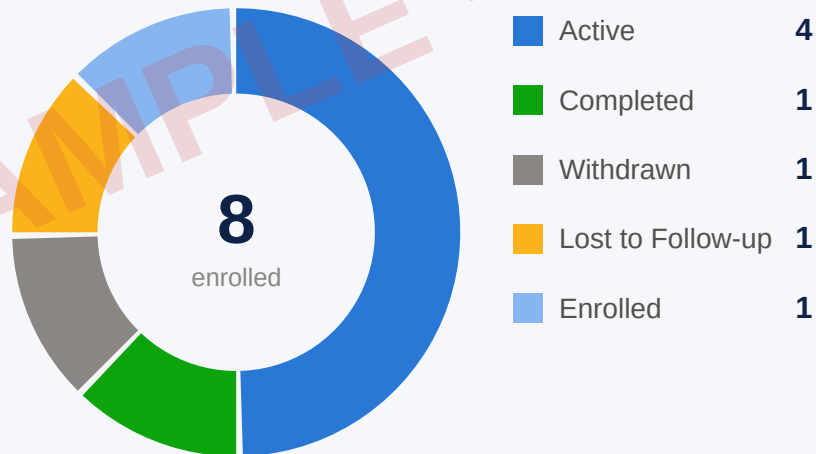
Each prescription carries the calculation trace that produced it: the inputs used, the weight basis, the rules that fired and the guideline behind them. Each batch carries its own code, manufacture and expiry date, and the patient can scan the pack for their own sheet.

None of this makes a single-arm pilot into evidence of effect — see the final page. It is what makes the numbers in between trustworthy as a record of what was actually prescribed and given.

The cohort

Who is in the pilot, and how far through it they are.

Status of every enrolled patient



7

With follow-up

at least one weekly review

40

Median days under care

5 with a recorded endpoint

27

Weekly reviews recorded

across the whole cohort

Every enrolled patient is counted here, including those who never received a product — the denominator is enrolment, not dispatch.

Weight

In advanced cancer the untreated trajectory is continued loss, so holding weight is itself a result.

86%

of patients held or gained weight

6 of 7 patients with paired measurements



4 gained 2 held 1 lost

Mean change +0.6 kg ±0.4 SE

Cohort mean weight

56 kg at enrolment → **56.6 kg at latest**

Change per patient

+2.2 kg



0

-2.2 kg

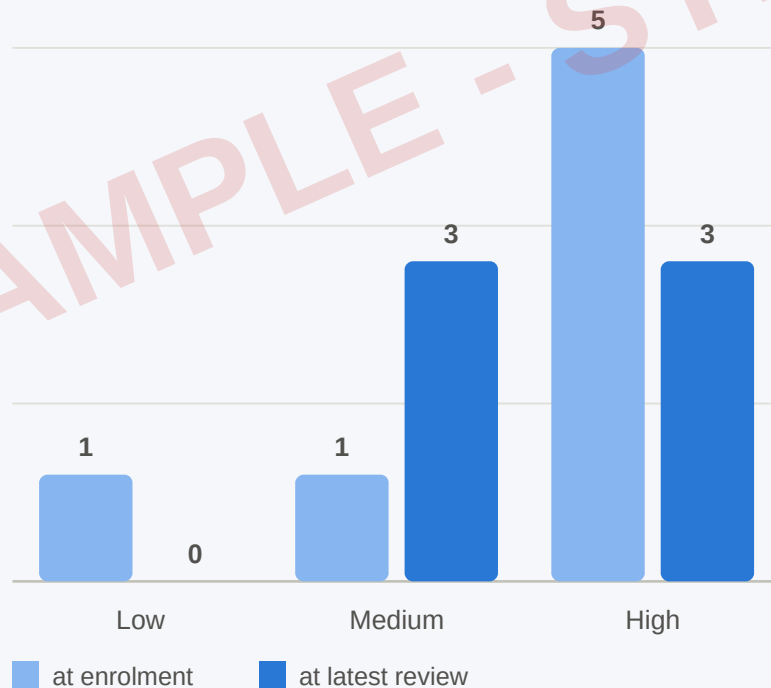
one column per patient, sorted

"Held" means within 2% of baseline — the band that separates real change from scale noise. Patients also received concurrent oncological treatment, which moves weight independently of nutrition.

Nutritional risk (MUST)

Screened at enrolment and rescreened at the most recent weekly review.

Patients in each risk band



Change per patient



Mean change -0.7 points

across 6 patients screened at both ends

A lower score is better. The change above, not the band counts opposite, is what moved.

MUST bands are coarse: 0 low, 1 medium, 2 or above high. A patient can improve by a point and stay in the high-risk band, so the change panel carries the signal, not the band counts. Weight loss is measured against usual weight six months before enrolment, the reference period MUST specifies. Patients whose screen could not be completed are excluded rather than scored as low risk.

Function and body composition

Cohort means, baseline against the latest review. Each row counts only patients with a value at both ends.



● baseline ● latest review

n differs by row: grip and MUAC are recorded less consistently than weight, so their denominators are smaller and their means correspondingly less certain. A mean over a handful of patients describes those patients; it is not an estimate of an effect.

Intake achieved against prescription

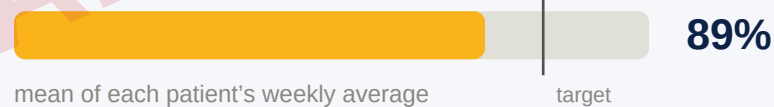
How much of the prescribed calorie and protein target patients actually took.

Cohort mean, as % of target

Calories



Protein



In absolute terms

1712 kcal

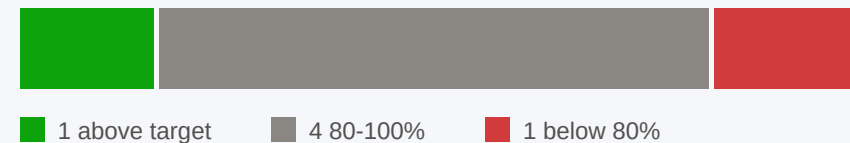
per day

78 g

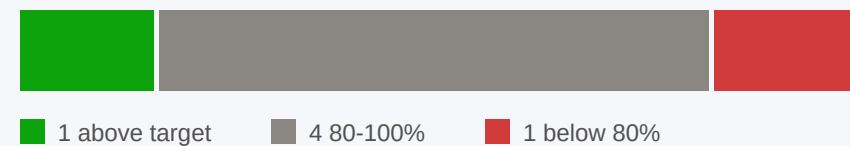
protein per day

Patients by target achieved

Calories



Protein



1 of 8 patients are not counted above

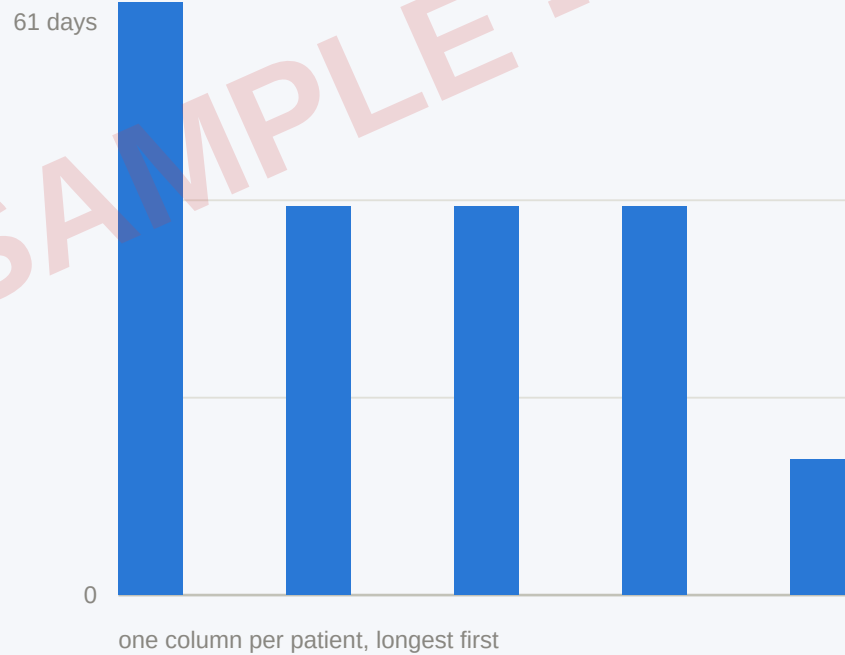
No week of theirs carries both a prescribed target and a recorded compliance figure, so there is nothing to estimate an intake from. They are left out rather than counted as zero.

Estimated, not measured: the prescribed calorie and protein split for each week, with the supplement portion scaled by the recorded compliance. Oral intake is a clinician's estimate rather than a weighed food record. Weeks with no compliance recorded are excluded rather than counted as zero. Adequate as a pilot signal; not a basis for a published intake claim.

Time under nutrition care

Enrolment to the most recent entry recorded for the patient.

Days under care, per patient with a recorded endpoint



40

Median days

5 with a recorded endpoint

39

Mean days

same denominator

3

Still under care

excluded from both figures

A patient still receiving care has no end date — their last entry is simply the most recent one, and their duration is still growing. Averaging them in with completed patients would understate how long the programme runs, so they are counted separately and the dataset marks their durations with a "+". This is not an inpatient length of stay: no admission or discharge date is recorded in this system.

How to read this

What these numbers can and cannot support.

Single-arm pilot, no control group.

Nothing here can be attributed to the supplement alone. Every patient also received oncological treatment, which independently moves weight, albumin, CRP and function.

n varies by measure.

A patient counts toward a measure only where both a baseline and a follow-up value exist. Grip and MUAC have smaller denominators than weight; each figure carries its own n.

Intake is estimated, not measured.

It is derived from the prescribed target and the recorded compliance. Oral intake is a clinical estimate, not a weighed food record.

MUST bands are coarser than the score.

Anything from 2 upward reads high risk, so a patient can improve without changing band. Read the per-patient change, not the band counts alone.

Incomplete screens are excluded, not scored zero.

A patient missing height or usual weight is reported as unscreened rather than counted as low risk.

Duration is censored.

Patients still under care are excluded from the duration figures rather than treated as finished.